



What is delirium?

To explain delirium in clear language and show why a sudden change needs urgent attention.

The short explanation

- Delirium is a sudden change in how the brain is working.
- It can affect attention, thinking, awareness, sleep, movement, mood and behaviour.
- Symptoms often change during the day. A person may seem clearer at one time and much more confused later.
- Delirium is a medical problem. It is not simply a normal part of ageing.

What it can look like

- The person may not follow a conversation, know where they are or recognise familiar people.
- They may be restless, frightened or unusually suspicious.
- They may see or hear things that are not there.
- They may instead become quiet, sleepy, withdrawn or slow to respond. This quieter form can be easy to miss.

Why it happens

Delirium usually means that one or more illnesses, medicines or other stresses are affecting the brain. Finding and treating these causes is central to care. Delirium and dementia are different, but a person can have both.

What good care involves

A trained healthcare professional should assess the person, look for possible causes and make an individual care plan. Calm explanation, familiar people, glasses or hearing aids, suitable food and fluids, safe movement, pain relief and support for sleep may all form part of that plan.

Urgent action New, sudden confusion needs immediate medical help. In the UK, call 999 or go to A&E now. Do not drive yourself to A&E. If the person is already in hospital or a care home, tell staff immediately. Outside the UK, call your local emergency number or use your local emergency service.

Verified sources

- [NHS: Sudden confusion \(delirium\)](#)
- [NHS inform: Delirium](#)
- [NICE CG103: Delirium recommendations](#)
- [SIGN 157: Risk reduction and management of delirium](#)

Use and review: UK-focused general education. This resource supports but does not replace clinical assessment, individual advice or current local guidance. Prepared from source materials curated by Professor Alasdair MacLulich, University of Edinburgh; AI-assisted production. Professor MacLulich led development of the 4AT and reports no financial interest in its uptake. Final named human clinical approval is required before use in patient care. Independent resource; not an official University of Edinburgh or NHS publication.